

Policy on Emergency Care Services

«Hospital Name»

Document control

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Prepared by	Emergency In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

«Hospital Name» provides emergency patients with accessible emergency care, appropriate resources, written emergency guidance, qualified triage, reassessment, documented disposition, discharge or transfer information, quality assurance, and defined processes for patients found dead on arrival or who die within a few minutes of arrival.

1. Purpose

The purpose of this policy is to establish requirements for receiving and managing emergency patients, triage, reassessment, disposition, transfer, emergency quality assurance, and management of patients found dead on arrival or who die within a few minutes of arrival.

2. Scope

This policy applies to the emergency department and personnel involved in emergency patient care at «Hospital Name».

3. Policy standards

3.1 Emergency area and resources

The identified area to treat emergency patients shall be easily accessible for initiation of care.

There shall be understandable signage and directions leading to the emergency area.

Emergency services preferably have designated triage, resuscitation treatment and patient holding areas.

The organisation shall specify the minimum number of beds based on its scope of services.

At a minimum, adequate and appropriate human resources, basic resuscitation equipment, equipment for monitoring vital parameters, appropriate consumables, and life-saving and emergency care drugs shall be available.

Personnel operating the emergency area shall be privileged to work in this area and have access to ongoing training.

3.2 Overcrowding and crowd management

Prevention of patient overcrowding includes monitoring footfall trends and developing strategies to manage overcrowding and violence.

High footfall times are expected to be anticipated, with adequate human resources assigned and strategies to manage overcrowding put in place.

Crowd management shall include an appropriate policy for patient relatives, attendants and visitors.

3.3 Emergency care and medico-legal cases

Written guidance shall include guidelines, SOPs or protocols for general emergency care and management of specific conditions, for example poisoning, road traffic accidents, acute stroke and coronary disease.

It shall address both adult and paediatric patients.

The procedure shall incorporate at a minimum identification, assessment and provision of care.

Where emergency services are outside the scope of the organisation, or facilities for appropriate emergency care of a given clinical condition are unavailable, at a minimum the patient shall be provided first aid before transfer to another organisation.

Processes shall be in place to ensure patient safety.

The organisation shall define what constitutes a medico-legal case (MLC).

Care provided, especially documentation and intimation to appropriate authorities, shall be in accordance with statutory requirements.

The organisation shall have a policy on management of suspected sexual assault and guidance on storage of samples of MLC patients.

3.4 Triage

Triage shall be done only by qualified/trained personnel.

Written guidance based on evidence or sound clinical practices shall guide triage.

Triage shall be part of routine day-to-day functioning of the emergency department and not only disaster management.

Criteria could be separate for trauma and non-trauma patients and adults and children, for example PAT or POPS.

If several clients are waiting to be triaged, a visual triage assessment may be conducted.

3.5 Reassessment

Patients waiting in the emergency are reassessed as appropriate for a change in status.

The findings of reassessment shall be documented in the patient's medical record.

3.6 Admission, discharge and transfer

The organisation shall maintain documentation indicating whether an emergency patient was sent home after initial care, admitted for further care, admitted in an emergency for a short stay and then discharged, or transferred to another organisation.

Staff shall have a clear understanding of the scope of the organisation's activities and the procedure for referral and transfer to another appropriate centre for patients who cannot be cared for in-house, after due first-aid or emergency care.

3.7 Discharge and transfer note

In case of discharge to home or transfer to another organisation, a discharge or transfer note shall be given to the patient.

The note shall contain salient clinical findings, investigations done, treatment given and condition at discharge or transfer.

The reasons for discharge or transfer shall be documented.

3.8 Quality assurance

The organisation shall implement a quality assurance programme.

Written quality assurance guidance may be developed individually or could be part of the organisation's overall quality-improvement programme.

The quality assurance programme shall involve all aspects of the functioning of the Emergency department.

The Emergency department shall collect data on key performance indicators, including care outcomes, as part of its quality improvement programme.

An example of a care outcome measure is return to the emergency department for the same complaint.

Collected data shall be collated, analysed and used for further improvements.

Improvements shall be monitored for sustenance.

3.9 Patients found dead on arrival

There shall be written guidance for managing a patient found dead on arrival, conforming to relevant local laws.

The guidance shall address maintaining a logbook of patients found dead on arrival; the decision on whether to perform a post-mortem; the decision regarding issue of a medical certificate of cause of death; temporary storage of the body in appropriate conditions; and what to do in case of unclaimed or unaccompanied bodies.

3.10 Patients who die within a few minutes of arrival

There shall be written guidance for managing a patient who dies within a few minutes of arrival after a failed attempt at resuscitation, conforming to relevant local laws.

The guidance shall address registration of such patients and recording the entire resuscitation events; the decision on whether to perform a post-mortem; temporary storage of the body in appropriate conditions; issue of a medical certificate of cause of death; and handing over of the body.

4. Non-negotiable rules

- Do not receive and treat an emergency patient in an area that is not the identified emergency area, or when triage-competent personnel and basic resuscitation equipment are not available.
- Do not leave overcrowding unmanaged when it blocks triage, resuscitation or safe holding.
- Do not provide emergency care without written guidance incorporating at minimum identification, assessment and provision of care.
- Do not transfer a patient without first providing first aid when appropriate emergency care is outside the organisation's scope or unavailable for the given clinical condition.
- Do not perform triage through personnel who are not qualified/trained.
- Do not treat triage as only a disaster-management activity; it shall be part of routine day-to-day emergency functioning.
- Do not omit documentation of reassessment findings in the patient's medical record.
- Do not discharge a patient home or transfer a patient without giving the patient the required discharge or transfer note.
- Do not omit any of the required contents of the discharge or transfer note or the reasons for discharge or transfer.
- Do not omit the required management processes for either a patient found dead on arrival or a patient who dies within a few minutes after arrival following failed resuscitation.

5. What we do

5.1 Emergency intake and resources

Maintain an accessible identified emergency area with understandable signage and directions.

Specify the minimum number of beds based on scope of services.

Maintain adequate and appropriate human resources, basic resuscitation equipment, vital-parameter monitoring equipment, consumables and life-saving/emergency care drugs.

Ensure emergency personnel are privileged to work in the area and have access to ongoing training.

5.2 Crowd management

Monitor footfall trends and develop strategies for overcrowding and violence.

Anticipate high footfall times, assign adequate human resources and put strategies in place.

Maintain an appropriate policy for patient relatives, attendants and visitors.

5.3 Emergency care

Use written guidelines, SOPs or protocols for general emergency care and specific conditions.

Cover adult and paediatric patients.

Use identification, assessment and provision of care as the minimum procedure elements.

Provide first aid before transfer when appropriate emergency care is outside scope or unavailable.

5.4 Triage

Ensure triage is performed only by qualified/trained personnel.

Use written guidance based on evidence or sound clinical practices.

Keep triage as part of routine day-to-day emergency functioning.

Separate criteria or visual triage assessment may be used as described in the Guidebook.

5.5 Reassessment

Reassess patients waiting in emergency as appropriate for a change in status.

Document reassessment findings in the patient's medical record.

5.6 Disposition and transfer

Document whether patients are sent home, admitted, kept for a short emergency stay and discharged, or transferred.

Ensure staff understand the scope of activities and referral and transfer procedures.

5.7 Discharge and transfer note

Give the patient a discharge or transfer note when discharged home or transferred.

Include salient clinical findings, investigations done, treatment given, condition at discharge or transfer, and reasons for discharge or transfer.

5.8 Quality assurance

Implement a quality assurance programme covering all aspects of Emergency department functioning.

Collect, collate and analyse key performance indicator data, including care outcomes.

Use collected data for further improvements and monitor improvements for sustenance.

The quality assurance programme may be standalone or could be part of the overall quality-improvement programme.

5.9 Patients found dead on arrival

Use written guidance conforming to relevant local laws.

Address the logbook, post-mortem decision, medical certificate of cause of death decision, temporary body storage and unclaimed/unaccompanied bodies.

5.10 Patients who die within a few minutes of arrival

Use written guidance conforming to relevant local laws.

Address registration and complete recording of resuscitation events, post-mortem decision, temporary body storage, medical certificate of cause of death and handing over of the body.

6. Stop-work authority

Do not receive and treat an emergency patient in an area that is not the identified emergency area, or when triage-competent personnel and basic resuscitation equipment are not available. Do not leave overcrowding unmanaged when it blocks triage, resuscitation or safe holding. Stop-work applies to unsafe emergency intake, not to immediate life-saving measures already under way — those continue while escalation happens. The person who stops tells the Emergency In-Charge and the Medical Superintendent the same shift. Refusing unsafe emergency intake is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Management	Management ensures the identified emergency area, required resources, written guidance and quality assurance arrangements.
Emergency In-Charge	The Emergency In-Charge oversees emergency operations and escalation of unsafe emergency intake.
Qualified/trained personnel	Qualified/trained personnel perform triage.
Emergency personnel	Emergency personnel follow applicable emergency, reassessment, referral, transfer and documentation processes.
Quality personnel	Quality personnel support quality assurance data collection, collation, analysis and improvement monitoring.

8. Quality monitoring

Monitoring area	What is monitored
Emergency footfall and overcrowding	Emergency department footfall and overcrowding trends
Emergency care KPIs	Emergency care key performance indicators, including care outcomes
Quality data	Quality data collation and analysis
Improvements	Further improvements and monitoring for sustenance
Reassessment and disposition	Reassessment documentation and patient disposition documentation
Discharge and transfer note	Discharge and transfer note requirements

9. Training and staff acknowledgement

Personnel operating the emergency area shall have access to ongoing training. Triage shall be performed only by qualified/trained personnel.

Staff shall have a clear understanding of the scope of the organisation's activities and applicable referral and transfer procedures.

I have read the Policy on Emergency Care Services of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to personnel involved in emergency care and emergency department operations.

11. Abbreviations

Abbreviation	Meaning
MLC	Medico-legal case
SOP	Standard Operating Procedure
PAT	Paediatric Assessment Triangle
POPS	Paediatric Observation Priority Score

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.2.a	Commitment	Sections 3.1 and 5.1 address the identified accessible emergency area, signage and directions, minimum beds, resources, privileged personnel and ongoing training. Stop-work authority (Section 6) also applies to this element.
COP.2.b	Achievement	Sections 3.2 and 5.2 address prevention of overcrowding and crowd management, including the required policy for patient relatives, attendants and visitors. Stop-work authority (Section 6) also applies to this element.
COP.2.c	CORE*	Sections 3.3 and 5.3 address emergency care in accordance with statutory requirements and written guidance, including minimum identification, assessment and provision of care, MLC definition, statutory documentation, sexual assault policy and MLC sample storage.
COP.2.d	Commitment*	Sections 3.4 and 5.4 address

		triage by qualified/trained personnel using written guidance as routine emergency functioning.
COP.2.e	Commitment	Sections 3.5 and 5.5 address appropriate reassessment for change in status and documentation of findings.
COP.2.f	Commitment	Sections 3.6 and 5.6 address documentation of admission, discharge and transfer and the clear referral/transfer process.
COP.2.g	Commitment	Sections 3.7 and 5.7 address the discharge/transfer note with salient clinical findings, investigations done, treatment given, condition at discharge/transfer and reasons.
COP.2.h	Achievement*	Sections 3.8 and 5.8 address the quality assurance programme, Emergency department coverage, KPI data, analysis, improvement and sustenance monitoring.
COP.2.i	Commitment*	Sections 3.9, 3.10, 5.9 and 5.10 address systems for management of patients found dead on arrival (five mandatory elements) and patients who die within a few minutes of arrival after failed resuscitation (four mandatory elements).

13. Required Records/Evidence Checklist

Emergency area and resources

- Identified emergency area with signage and understandable directions.
- Minimum number of beds specified based on scope of services.
- Human resources, basic resuscitation equipment, vital-parameter monitoring equipment, consumables and life-saving/emergency drugs available.
- Emergency personnel privileging and ongoing training arrangements.

Crowd management

- Footfall trend information and overcrowding/violence management strategies.
- Planning for high-footfall times and allocation of adequate human resources.
- Policy for patient relatives, attendants and visitors.

Emergency care and MLC

- Written emergency guidelines, SOPs or protocols for general care and specific conditions.
- Adult and paediatric emergency guidance.
- Emergency procedure covering identification, assessment and provision of care.
- First-aid and transfer arrangements where appropriate emergency care is outside scope or unavailable.
- Patient-safety processes.
- Definition of MLC and statutory documentation/intimation process.
- Policy for suspected sexual assault management and guidance for storage of MLC samples.

Triage and reassessment

- Qualification/training arrangements for personnel performing triage.
- Written triage guidance based on evidence or sound clinical practices.
- Routine day-to-day triage process.
- Medical records containing documented reassessment findings.

Disposition and transfer

- Emergency records showing whether patients were sent home, admitted, kept for short emergency stay and discharged, or transferred.
- Referral and transfer procedure for patients who cannot be cared for in-house after due first-aid/emergency care.
- Discharge/transfer notes containing salient clinical findings, investigations done, treatment given, condition at discharge/transfer and reasons for discharge/transfer.

Quality assurance

- Written quality assurance guidance, standalone or part of the overall quality-improvement programme.
- Emergency department quality assurance covering all aspects of functioning.
- Key performance indicator and care-outcome data with collation and analysis.
- Improvement actions and monitoring for sustenance.

Found dead on arrival

- Written guidance conforming to relevant local laws.
- Logbook for patients found dead on arrival.
- Post-mortem decision and medical certificate of cause of death decision processes.
- Temporary body storage arrangements.

- Process for unclaimed or unaccompanied bodies.

Death within a few minutes of arrival

- Written guidance conforming to relevant local laws.
- Registration process and complete recording of resuscitation events.
- Post-mortem decision process.
- Temporary body storage arrangements.
- Medical certificate of cause of death process and body-handover process.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. COP.2.
2. Guidebook interpretation supplied for COP.2.a through COP.2.i.
3. Relevant local laws for management of patients found dead on arrival or who die within a few minutes of arrival.

Disclaimer

This policy is a plain-language reorganisation of the supplied COP.2 objective elements and Guidebook interpretation. Mandatory requirements, stated examples, preferable measures and optional mechanisms have been kept at their stated level of strength.